



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : امال جابر حسين سليمان

العنوان : ممممممممممممممممممممم

المحافظة : الاسكندرية الحي / المدينة : الجمرك

تاريخ تسجيل الإستمارة : 19-09-2026

رقم البطاقة : 25711182101506 رقم التليفون : 01129998588

العمر (تاريخ الميلاد) : 18-11-1957 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة في هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة رئيس اللجنة ومدير المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... د/..... د/..... د/..... د/.....

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: امال جابر حسين سليمان National ID: 25711182101506	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الى Supportive and palliative Treatment
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

Patient's Name: Amal Jaber Hussein
Date: 2026-06-20
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS: -

Compared to attached study report dated 15-11-2025, the current study shows:

I Chest:

- Mildly enlarged size regarding the previously seen left breast lesions largest measuring 3.7 X 2.7 cm in today's study, as well as the left axillary lymph nodes the largest measuring 2.1 x 1.8 cm in today's study.
- Rather stationary course regarding the previously seen left supraclavicular lymph nodes.
- Progressive course regarding the size and number of the previously noted multiple bilateral scattered pulmonary nodules largest measures 12.7 x 10 mm at the left lower lung lobe.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Bilateral elastofibroma dorsi more sizable on the left side.

II Abdomen & pelvis:

- Enlarged liver with homogenous low CT texture. Still no focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- The examined skeleton revealed:
 - Newly developed manubrium sterni lytic bone lesion.

Patient's Name: Amal Jaber Hussein
Date: 2026-06-20
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

- Stationary course regarding the previously seen left iliac heterogenous mixed lytic and sclerotic lesion.
- Still noted healing sternal fracture.
- Left hip osteoarthritic changes with geodes.
- Dorsolumbar spondylosis and scoliosis.

Opinion:

- Progressive course regarding the previously seen left breast lesion, left axillary lymph nodes, pulmonary nodules as well as the newly developed manubrium sterni lytic bone lesion.
- Stationary course regarding the rest of the study.

Dr.sherif Wagdy

Dr.sherif Wagdy

04/06/2026

MDT FORM

Name:	Amal Jaber Huseen .	MRN:	363
Age:	68 Y	ID:	25711182101506
Menopausal Sta	postm	Phone:	01129998588
NRS SCORE:	ps 1	Marital Status:	
BMI:		Family History:	
CO Morbidity:		Reason Of MDT:	

Clinical Data:

BX: Idc
ER: 8/8 PR: 4/8 HER2: 3+ KI67:

R

L

Sonomammography:

Mets Workup:

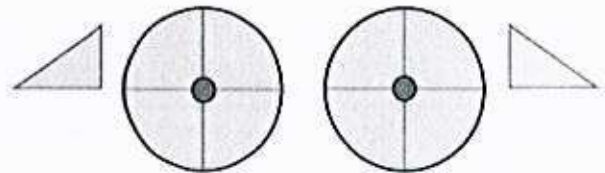
History:

68 Y POSTM LEFT BREAST CANCER MET TO BONE ER8/8 PR4/8 HER23+
DIAGNOSED IN 2022 ST FEMARA AND HERCEPTIN TILL 1/2025 PD ST XELODA
TYKERB .CTS 11/2025 INCREASE SIZE OF PUL NODULE 9 MM VS PREVIOUS 2
TO 3 MM bone scan on 9/3/2026 comp to bs 4/2025 from 3/2025 mild
progression of metastatic bone deposits ,sd of osseous lesions with newly
developped osseous lesion at sternum and left 6th rib for mdt(pt st xeloda
tykerb from 3/2025 and tykerb stopped for few months bec of approval)

Recommendations:

MDT Outcome & Plan

for navelbine + - tykerb (depending on approval)



T N M

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
Dr.henan ezzat					

جمهورية مصر العربية
بطاقة تحقيق الشخصية

أمال
جابر حسين سليمان
١٨ محمد صبحي المحامي - أرض اللواء
العجوزة - الجيزة

٢٥٧١١١٨٢١.١٥.٦

KJ9459820

٢٠٢٤/٠٢ ٢٥٧١١١٨٢١.١٥.٦

رربة منزل

أنثى مسجلة أرملة

البطاقة سارية حتى ٢٠٣١/٠٢/٠٦

١



SCAN 8 LABS

Name : Amal Jaber Hussein Suleiman

Branch : Dokki

Ref.Physician Prof : Goustav Roussey Egypt

مركز النيل

Date : 09-03-2026 08:37

Medical No. : 329545

Ins.Company : تعاقد مستشفى

ISOTOPIC BONE SCAN REPORT

- **Indication:** 68-year-old female with history of metastatic breast carcinoma; follow-up assessment is requested. Prior bone scan dated 15/04/2025 demonstrated few osseous lesions, likely metastatic in nature.
- **Radioisotope:** Tc-99m MDP
- **Imaging Protocol:** Whole-body planar images (anterior and posterior projections) acquired approximately 2–3 hours post intravenous injection of the tracer

FINDINGS:

In comparison with the prior bone scan dated 15/04/2025, the current study demonstrates:

- **Mildly regressed** tracer uptake intensity at the known osseous lesion involving the **left acetabulum**.
- **Stationary course** of the known osseous lesions at the posterior aspect of the **left 7th rib** and anterior aspect of the **right 4th rib**.
- **Newly detected osseous lesions** at the **sternum** and anterior aspect of the **left 6th rib**, indicating disease progression.
- Degenerative/arthritis changes are noted at multiple levels of the spine (with associated lumbar scoliosis), both shoulders, and the weight-bearing joints.

IMPRESSION:

- **Mild progression of metastatic bone disease** since the prior study dated 15/04/2025 as above-detailed.
- Clinical and imaging follow-up is recommended as clinically indicated.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine

A-Patient ID:
1- Name : امان جابر حسين شليمان

2-ID number:

Sex:

MDT form

3- Medical number:
Female

83047945
Old Med :

Old Med :
5380-2021

Patient Age : 67 y

Family History

marital status

Co morbidity

Phone

NRS Score

Menopausal status

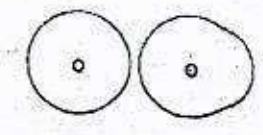
Phone

01142430699

BMI

GROUP

Initial STAGE



REASON OF MDT

REASON OF MDT

Date of MDT

16/02/2025

Breast Imaging :			Clinical data		
			Mets Workup :		
BX IDC	ER 8/8	PR 4/8	Her 2 Neu	SISH	KI67
T			M		
			1 bone		

Sonomammography

PATIENT HISTORY

MBC to bone

pt received herceptin iv x 27 cycle+AI last 11/2023

kept under AI+zometa till now

12/2024 CTCAP , BSM : progressive breast lesion , stationary bone mets

bone_scan requested & pending

MDT OUTCOME & PLAN

for 2nd line Xeloda / tykerb x 3 mns then reass.

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY	RADIOLOGY	RADIOLOGY
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition

Patient Name : الأستاذة / امال جابر حسين سليمان

Patient No. : 25711182101506

Path No. : SR-743/21

Sex : Female

Age : 64 Y

Referring Doc :

Request No. : 133527

Request Date : 20/12/2021

Verification Date : 28/12/2021

Financial Category : نفقة الدولة - اورام دار السلام

Department : Out Patient

Registered by : .marwa mohamed youss

Pathology Report**Clinical Data:**

Left breast mass

Nature of the specimen:

Slide revision (Tru-Cut biopsy)

Gross Examination:

1 block (9332/21) & 3 slides

Microscopic examination:

- Histological type: Invasive duct carcinoma grade II
- Angioinvasion: No
- Ductal carcinoma in situ: No
- Lymph node involvement: N/A
- Resection Margins: N/A
- Histologic response to therapy: N/A

Diagnosis:

Left breast mass; Slide revision:

Invasive duct carcinoma grade II

IHC:

ER: Strong nuclear reaction in 97% of tumour cells. Positive, score (8/8)

PR: Moderate nuclear reaction in 20% of tumour cells. Positive, score (4/8)

Her-2: Strong complete membrane reaction in 10% of tumour cells. Positive, score 3+

Prof. Dr. Amal Hareedy

Dr. Abdulrahman Ibrahim

Verified by :

abdelrahman ibrahim

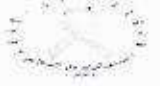
Printing Date : 11/02/2025 10:13:14

Page 1 of 2

Printing By : .marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name الأستاذة / امال جابر حسين سليمان
Patient No. 25711182101506
Path No. SR-743/21
Sex Female
Age 64 Y
Referring Doc.

Request No. 133527
Request Date 20/12/2021
Verification Date 28/12/2021
Financial Category اورام دار السلام - نفقة الدولة
Department Out Patient

Registered by .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Verified by :
abdelrahman ibrahim

Printing Date : 11/02/2025 10:13:20

Page 2 of 2

Printed By : .marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



SCAN & LABS

Name : Amal Jaber Hussein Suleiman

Branch : Dokki

Ref.Physician Prof : Goustav Roussey Egypt

Date : 09-03-2026 08:37

Medical No. : 329545

Ins.Company : تعافد مستشفيات

مركز النيل

ISOTOPIC BONE SCAN REPORT

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- Degenerative/arthritis changes are noted at multiple levels of the spine (with associated lumbar scoliosis), both shoulders, and the weight-bearing joints.

IMPRESSION:

- **Mild progression of metastatic bone disease** since the prior study dated 15/04/2025 as above-detailed.
- Clinical and imaging follow-up is recommended as clinically indicated.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine



Patient Name: Amaal Gaber Hussein Slimin	Date : 11/02/2025
Exam : MAMMOGRAPHY BOTH	Age : 67 Year(s)
Medical ID: 5386/2021	Pat. ID: 18620

**DIGITAL BILATERAL SOFT TISSUE MAMMOGRAPHY AND
COMPLEMENTARY ULTRASOUND**

Indication:

Patient is 67 years old and is seen for metastatic left breast malignant mass

Report:

- ACR c: heterogeneous fibro-glandular parenchyma of both breasts
- Left Breast:
 - A dense spiculated irregular mass is seen in the left UOQ measuring 45x40x35mm about 40mm from the nipple and attached to the overlying skin.
 - Ultrasound shows multiple smaller suspicious masses in the UOQ and in the lower inner quadrant at 7O'clock zone c measuring 9x8mm.
 - Multiple (>3) pathologically enlarged level I axillary lymph nodes. The largest is measuring 27x16mm.
 - No levels II or III axillary or supraclavicular LNS
- Right Breast:
 - No suspicious micro calcifications or architectural distortion detected.
 - A well-defined oval parallel hypoechoic mass is seen at 10'clock measuring about 11x7mm.
 - Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
 - Nonspecific axillary LN.

Opinion:

- Metastatic left breast malignant mass with multicentric disease and ipsilateral axillary lymphadenopathy (BIRADS 6)
- Probably benign findings of right breast (BIRADS 3)

Best regards,
Dr. Amira H Radwan MD,
Breast Imaging consultant,
NCI, Cairo University
11/02/2025 1:15 PM



Total DLP (mGy.cm):

Patient Name: Amaal Gaber Hussein Slimin	Date : 20/01/2024
Exam : C.T CHEST WITH CONTRAST	Age : 66 Year(s)
Medical ID: 5386/2021	

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN AND PELVIS: -**

**Compared to attached study report dated 24/01/2023, the
current study shows:**

I Chest:

- Stationary course regarding the left breast lesions largest measures 1.6 x 1.8 cm and left axillary level I & II subcentimetric lymph nodes.
- No sizable pulmonary nodules, masses or consolidations.
- Still noted bilateral basal ground glass faint attenuations.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Bilateral elastofibroma dorsi more sizable on the left side.

II Abdomen & pelvis:

- Enlarged liver with homogenous low CT texture. Still no focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.



Total DLP (mGy.cm):

Patient Name: Amaal Gaber Hussein Slimin	Date : 20/01/2024
Exam : C.T CHEST WITH CONTRAST	Age : 66 Year(s)
Medical ID: 5386/2021	

- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- The examined skeleton revealed:
 - Stationary left iliac lytic lesion heterogenous sclerotic lesion.
 - Left hip osteoarthritic changes with geodes.
 - Dorsolumbar spondylosis and scoliosis.

Opinion:

- Follow up study showing overall stationary course.

Dr.Shaimaa Megahed,MSc

Dr. Ofa Magdy, M.D.

Kasr El-Ainy Center of Radiation Oncology and
Nuclear Medicine

Faculty Of Medicine, Cairo University

"NEMROCK"

Nuclear Medicine Unit



مركز قصر العيني

لعلاج الأورام والطب النووي
كلية الطب - جامعة القاهرة
(ممرات)

Report

Patient Name:	Amal Gaber Hussein Soliman	Patient Gender:	Female
Patient Code:	Q44547	DATE:	4/15/2025 10:31:58 AM

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

Clinical history

- o 68-year-old female patient with history of left breast cancer.

Technique

- o Anterior and posterior whole body scans were acquired 2 hours post IV injection of 740 MBq of the tracer.

Findings

- o Active tracer uptake is noted in the left acetabulum, right 4th rib anteriorly and left 10th rib posteriorly.
- o Arthritic changes of the knees and shoulders are noted.
- o Patchy tracer uptake is noted in the spine more pronounced in lumbar region? Degenerative? Metastatic, for MRI confirmation to justify its nature.
- o No other serious bone lesions detected.

Opinion

- o Few osseous lesions as described likely metastatic in nature.

Dr. Hagar Nageh

Dr. Nahla Dessoki, MD

